Motorized Spinal Traction

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➲ Instructions for Use

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Related Commercial/Individual Exchange Policies

- Electromagnetic Therapy for Wounds
- Home Traction Therapy
- Mechanical Stretching Devices

Community Plan Policy

- Motorized Spinal Traction

Application

UnitedHealthcare Commercial

This Medical Policy applies to UnitedHealthcare Commercial benefit plans.

UnitedHealthcare Individual Exchange

This Medical Policy applies to Individual Exchange benefit plans.

Coverage Rationale

Motorized spinal traction devices are unproven and not medically necessary for treating neck and low back disorders due to insufficient evidence of efficacy.

Applicable Codes

The following list(s) of procedure and/or diagnosis codes is provided for reference purposes only and may not be all inclusive. Listing of a code in this policy does not imply that the service described by the code is a covered or non-covered health service. Benefit coverage for health services is determined by the member specific benefit plan document and applicable laws that may require coverage for a specific service. The inclusion of a code does not imply any right to reimbursement or guarantee claim payment. Other Policies and Guidelines may apply.

HCPCS Code	Description
S9090	Vertebral axial decompression, per session

Description of Services

Vertebral axial decompression is a type of spinal traction used in the treatment of back or neck pain.

This involves the use of a computer-driven table to control the disc decompression. For the treatment, a pelvic harness is applied to the patient and the patient lies on the special table and is subjected to a series of cycles as the table is slowly extended and a distraction force is applied via the harness. When the desired tension is reached, it is gradually decreased. The number of sessions varies.

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Clinical Evidence

Back

There is insufficient evidence from peer-reviewed published studies to conclude that spinal unloading devices are effective in the management of low back pain (LBP) or that they improve health outcomes. Available studies have limitations such as small sample size, incomplete data, lack of follow-up after therapy ceased or short-term follow-up. Additional well-designed controlled trials are needed to determine the efficacy for this service.

Adar et al. (2023) conducted a retrospective study aimed to compare the effectiveness of motorized traction and non-surgical spinal decompression (NSD) with other treatment options such as conventional motor traction in the treatment of LBP caused by lumbar discopathy. Individuals diagnosed with lumbar discopathy who underwent physical therapy in author's clinic were reviewed. Demographic data, duration of their symptoms, physical examination findings, lumbosacral magnetic resonance imaging (MRI) reports, method and duration of treatment, and Visual Analog Scale (VAS) and Oswestry Disability Index (ODI) results were recorded. A total of 160 patients met the inclusion criteria. Their mean age was 44.6 ±12.4 (range 21-65) years, 57.5% (n = 92) were female, and 42.5% (n = 68) were male. There were no differences between the conventional physiotherapy, motorized traction, and spinal decompression groups in terms of age, duration of symptoms, and the number of sessions ($p > 0.05$). In all three groups, the mean scores of VAS and ODI were decreased in the pre-and post-treatment comparisons ($p < 0.005$). The rates of change in VAS and ODI were higher in the traction group and spinal decompression group compared to the conventional treatment ($p < 0.005$). The authors concluded that patients with subacute and chronic lumbar discopathies, motorized traction and spinal decompression treatments added to conventional treatment were found to be more effective than conventional treatment alone. The results of spinal decompression and conventional motorized traction (CMT) treatments appear to be similar. Limitations of include the retrospective nature of the study, the heterogeneity of the patient groups and the small sample size. In addition, factors such as body mass index, accompanying fibromyalgia, drug use, different decompression levels, exercise compliance, lifestyle factors, or psychological factors (e.g., fear-avoidance beliefs, anxiety about pain), which have the potential to affect the effectiveness of treatments and results were not evaluated may also have affected the results. The exclusion criteria might limit the generalizability of the study to only patients with specific characteristics, reducing the diversity of the sample. Due to these limitations, it has insufficient results in terms of clinical acceptance. Randomized controlled trials (RCTs) with balanced group sizes would provide more reliable comparisons between the treatments.

Vanti et al. (2023) conducted a systematic review and meta-analysis of RCTs comparing the effects of different types or parameters of lumbar traction in LBP. Methods: CENTRAL, CINAHL, ISI Web of Science, PEDro, PubMed, and Scopus databases were searched from their inception to March 31, 2021. All RCTs comparing different types or parameters of lumbar traction on adults who complained of LBP with or without lumbar radiculopathy (LR) were considered. Any restriction regarding publication time or language was applied. Two reviewers independently selected the studies, performed the quality assessment, and extracted the results. Meta-analysis employed a random-effects model. Sixteen studies met the inclusion criteria for qualitative analysis, and five were pooled. Meta-analyses of results from five studies on LBP with LR showed no difference between diverse tractions modalities at short-term follow-up. Very low to low-quality evidence supports these results. High-force and low-force traction demonstrated improvements in pain. The authors concluded that the literature suggests short-term effectiveness of traction on pain in LBP with LR, regardless of the type or the dosage employed. Different effects of traction other than mechanical ones can be hypothesized. This systematic review may be relevant for clinical practice due to the similar effects of different traction types or dosages. The small number of studies included in quantitative synthesis is the most important limitation of this review. Very often, no information was reported about dropouts, and even if they are reported, the data related to the participants who have not completed the study were not specified. Several studies were excluded due to incomplete data on the outcomes, mostly when only percentages of improvement or worsening were reported, instead of pre- and post-treatment means and standard deviations. A small sample size makes it difficult to decide whether these conclusions can be generalized to a larger population. Further investigation is needed before clinical usefulness of this procedure is proven.

Amjad et al. (2022) conducted an RCT to determine the effects of NSD therapy in addition to routine physical therapy on pain, lumbar range of motion (ROM), functional disability, back muscle endurance (BME), and quality of life (QOL) in patients with radiculopathy. A total of 60 patients with LR were randomly allocated into two groups, an experimental (n = 30) and a control (n = 30) group, through a computer-generated random number table. Baseline values were recorded before providing any treatment by using VAS, Urdu version of Oswestry disability index (ODI-U), modified-modified Schober's test (MMST), prone isometric chest raise test, and Short Form 36-Item Survey (SF-36) for measuring the pain at rest, functional disability, lumbar ROM, BME, and QOL, respectively. All patients received 12 treatment sessions over four weeks, and then all outcome measures were again recorded. By using the ANCOVA test, a statistical ($p < 0.05$) between-group improvement was observed in VAS, ODI-U, BME, lumbar ROM, role physical (RP), and bodily pain (BP)

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domains of SF-36, which was in favor of NSD therapy group. The between-group difference was 1.07 ±0.32 cm ($p < .001$) for VAS, 5.65 ±1.48 points ($p < .001$) for ODI-U, 13.93 ±5.85 s ($p = 0.002$) for BME, 2.62 ±0.27 cm ($p < .001$) for lumbar flexion, 0.96 ±0.28 ($p < .001$) for lumbar extension, 5.77 ±2.39 ($p = 0.019$) for RP and 6.33 ±2.52 ($p = 0.016$) for BP domain of SF-36. For these outcomes, a medium to large effect size (d = 0.61-2.47, 95% CI: 0.09-3.14) was observed. The authors concluded that a combination of NSD therapy with routine physical therapy is more effective, statistically, and clinically, than routine physical therapy alone in terms of improving pain, lumbar ROM, BME, functional disability, and physical role domain of QOL, in patients with LR, following four weeks of treatment. Limitations to this RCT include additional therapy time given to the interventional group compared to the control group. The "high-technology" intervention and additional therapy time vs control may have significantly impacted patient-reported outcome measures (PROMs) and led to the potential Hawthorne effect. Due to the nature of the treatment, it was not possible to maintain patients' blinding, which may also have caused the Hawthorne effect. In addition, the lack of follow-up after therapy ceased was another limitation. The short-term follow-up did not allow for assessment of intermediate and long-term outcomes.

A random cross over study performed by Lee et al. (2021) evaluated real-time standard spinal traction (ST) with that of lordotic curve-controlled traction (LCCT). The study included 40 participants with mild non-radicular LBP and randomly assigned for either standard ST or LCCT. Each participant had initial x-rays taken in a standing position. After ten minutes of traction, another radiograph was taken in the supine position, and real-time shooting was performed during both standard ST and LCCT procedures. The following angles were measured: intervertebral disc angle of all segments, disc distance anterior and posterior and all measurements were taken by a radiologist who was blinded to the study. The disc distance was defined as the distance between inferior endplate of upper vertebrae and the superior endplate of opposing lower vertebrae while applying standard ST to straighten the spine or LCCT to be applied posteriorly to maintain the lordotic curve. Standard ST was applied and gradually increased to the maximum level tolerated or until the force was 1/3 of the patient's weight. LCCT participants had a magnetic marker attached to L4/L5 disc space by physical palpation. The authors found that during standard ST the force of traction decreased the lordotic curve and had more effect on the posterior and overstretching which causes pain, muscle spasms, damage to facet joints and soft tissue without effect on discs. The LCCT group with the same amount of force showed greater distance increase in discs and fewer muscle spasms. The authors concluded that the LCCT preserved the lordotic curve whereas standard ST only straightened it. The authors felt the newly developed LCCT device was useful for increasing the disc space evenly while maintaining the lordotic curve. Limitations included small sample size and lack of long-term efficacy for low LBP; further studies are warranted.

Tanabe et al. (2021) performed an RCT to evaluate the efficacy and safety of traction on chronic low back pain (CLBP) patients using recently developed equipment capable of precise traction force control. The study included 95 patients with non-specific CLBP from 28 clinics and hospitals, distributed throughout Japan, between December 2016 and March 2017. Participants were randomly assigned to group A (n = 49), intermittent traction with vibration (ITV) mode; and group B (n = 46), intermittent traction only (ITO) mode. All patients were followed up weekly for two periods after study-initiation. The primary outcome measures were disability level including pain, and QOL. Statistical analysis was performed using linear mixed model. Two types of traction devices sold in the market under the same category of classification (MINATO Medical Science, ST-2L/2CL and OG Wellness Technologies, OL-6500/6000) were used. The devices consist of two main parts: a holding part for the upper body with arm holders, and a computerized moving part for the lower body. The upper body unit automatically measures the height of the arm pit to maintain the counter force against traction. The lower body unit produces a position of 90/90° traction adjusting the thigh length. Comparing to pre-traction data, both traction modes showed improvement except the first intervention of ITO treatment. The differences in Japan Low Back Evaluation Questionnaire (JLEQ) scores over time showed improvements in the treatment to which vibrational force was added in contrast to the conventional traction treatment; the MD was significant to compare ITV treatment and ITO treatment (−1.75 ($p = 0.001$), 95% CI; −2.69 to −0.80). However, neither difference between the two sequences ($p = 0.884$) nor carryover effect ($p = 0.527$) was observed. The authors concluded that lumbar traction could provide immediate effect in terms of the pain intensity and functional status in patients with CLBP, and a traction method added vibrational force on preload seemed to be promising. In addition, the study contributes to some evidence of the efficacy of lumbar traction. Limitations of the study include a short follow-up period of two weeks which did not allow for assessment of intermediate and long-term outcomes. Further investigation is needed before clinical usefulness of this procedure is proven.

A systematic review with meta-analysis was completed by Colombo et al. (2020) to investigate the effectiveness of traction therapy in reducing pain in patients with cervical radicular syndrome (CRS). Two reviewers independently selected RCTs that compared traction in addition to other treatments versus the effectiveness of other treatments alone for pain outcome. The authors calculated the MDs and 95% confidence intervals (CIs). They used Cochrane's tool to assess risk of bias and the Grading of Recommendations Assessment, Development and Evaluation (GRADE) system to evaluate the quality of evidence and summarize the study. A total of seven studies (589 patients), one with low risk of bias, were evaluated. An overall estimate of treatment modalities showed low evidence that adding traction to other treatments is statistically significant [MD −5.93 (95% CI, −11.81 to −0.04) $p = 0.05$ and $I2 = 57\%$] compared to other

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treatments alone. The subgroup analyses were noteworthy only for mechanical and continuous modalities. The authors concluded overall analysis showed that, compared to controls, reduction in pain intensity after traction therapy was achieved in patients with cervical radiculopathy. However, the quality of evidence was generally low and none of these effects were clinically meaningful. This systematic review with meta-analysis has several imitations. The authors did not investigate other functional outcomes (e.g., abilities of daily living) or adverse events. To obtain a broader view of different traction techniques, the authors included a wide variety of control groups which may have reduced the accuracy of comparisons. The available evidence is limited with overall poor-quality methodology and design. Therefore, no conclusions can be made regarding the relative efficacy, effectiveness, or safety of treatment. Future RCTs should investigate other interventions for CRS, apply homogeneous and universally accepted inclusion criteria and clinical examinations, focus on patients with acute symptoms and adopt explicit methods to minimize selection, performance, and detection bias.

Cheng et al. (2020) completed a systematic review of seven articles and a meta-analysis of literature including 403 participants. The criteria assessed in the RCT included participants with LBP (with or without sciatica), and those with herniated disc(s) confirmed by MRI or computed tomography (CT). The analysis compared participants that received any type of traction to the lumbar spine with sham or no traction and pain measurements before and after intervention. The authors concluded that lumbar traction was effective in the short term for reducing LBP in those with a lumbar herniated disc, but further studies are needed to determine long term effectiveness. Several limitations of the study were identified including methodology, small sample size, differing interventions and outcome assessments contributing the heterogeneity; in addition, only two trials used sham controls.

Tadano et al. (2019) conducted a qualitative study as part of an RCT (UMIN-CTR 000024329, date opened: October 13, 2016) to examine the biomechanical change at the lumbar area under lumbar traction and confirm its reproducibility and accuracy as a mechanical intervention. A total of 133 patients with non-specific CLBP from 28 orthopedic clinics to assess and determine traction conditions while undergoing a biomechanical experiment. Two types of commercially available motorized traction devices (MINATO Medical Science, ST-2 L/2CL and OG Wellness Technologies, OL-6500/6000) were used and incorporated into other measuring tools including an infrared rangefinder and large extension strain gauge. The finite element method (FEM) was used to analyze the real data of pelvic girdle movement at the lumbar spine level. Self-report assessments with representative two conditions were analyzed according to the qualitative coding method. Thirty-eight participants provided available biomechanical data. Distraction force lineally correlated with the movement of traction unit at the pelvic girdle. After applying vibration force to preloading, the strain gauge showed proportional vibration of the shifting distance without a phase lag qualitatively. FEM simulation provided at least 3.0-mm shifting distance at the lumbar spine under 100 mm of body traction. Ninety-five participants provided a treatment diary and were classified as no pain, improved, unchanged, and worsened. Approximately 83.2% of participants reported a positive response. The authors concluded that the current study, which combined a biomechanical experiment with FEM simulation and analysis of patients' perspective, found that lumbar traction operates as an actual mechanical intervention therapy for patients with CLBP, and it provided the possibility of an immediate effect after traction. The identification of an appropriate loading mode, a limitation to this study, may still be an essential step for ascertaining the clinical utility of lumbar traction. In addition, only the distance on the lumbar skin was assessed rather than direct assessment of the shift of discs or vertebral bodies. The findings of this study need to be validated by well-designed studies. Further investigation is needed before clinical usefulness of this procedure is proven.

Koçak et al. (2017) studied and compared the efficiency of CMT with NSD using the DRX9000TM device, a different form of motorized spinal traction, in patients with LBP associated with lumbar disc herniation. Forty-eight patients were randomized into two different groups; the first group underwent CMT, and the second group underwent NSD. Both groups underwent the therapy for six weeks. Participants were assessed before and after the sessions: pain was assessed by using VAS, functional status assessed using ODI, and QOL assessed using the Short Form-36 (SF-36), state of depression mood assessed using the Beck Depression Inventory (BDI), and the global assessment of the illness using the Patient's Global Assessment of Response to Therapy (PGART) and Investigator's Global Assessment of Response to Therapy (IGART) scales. The authors concluded the study findings showed both CMT and NSD treatments were effective methods in controlling pain, in enhancing functional status, and in reducing depressive mood in patients with CLBP associated with LDH. Limitations included lack of control group without motorized spinal traction, no sham groups, and the inability to perform long-term follow-up of the participants; future studies are warranted.

In an RCT, Thackeray et al. (2016) examined the effectiveness of mechanical traction in patients (n = 120) with LBP and nerve root compression. Patients were randomized to receive an extension-oriented treatment approach with or without the addition of mechanical traction, and over a six-week period, patients received up to 12 treatment visits. Primary outcomes of pain and disability were collected at six weeks, six months, and one year by assessors blinded to group allocation. At the end of the one-year time frame, the authors concluded that in this patient population there was no

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evidence that mechanical lumbar traction in combination with an extension-oriented treatment was superior to extension-oriented exercises alone in the management of these patients at any point in the evaluation period.

Wegner et al. (2013) conducted a systematic review to determine if traction was more effective than reference treatments, placebo, sham traction, or no treatment for LBP with or without sciatica, with a focus on pain intensity, functional status, global improvement and return to work. The authors included RCTs using traction, including mechanical traction, manual traction (unspecific or segmental traction), computerized traction, auto-traction, underwater traction, bed rest traction, inverted traction, continuous traction, and intermittent traction. This is an update of a Cochrane review first published in 1995 and previously updated in 2006. This systematic review included a total of 32 RCTs involving 2,762 participants. For people with mixed symptom patterns (acute, subacute and CLBP with and without sciatica), there was low- to moderate-quality evidence that traction may make little or no difference in pain intensity, functional status, global improvement or return to work when compared to placebo, sham traction or no treatment. When comparing the combination of physiotherapy plus traction with physiotherapy alone, or when comparing traction with other treatments, there was very-low- to moderate-quality evidence that traction may make little or no difference in pain intensity, functional status, or global improvement. For people with LBP with sciatica and acute, subacute, or chronic pain, there was low- to moderate-quality evidence that traction probably has no impact on pain intensity, functional status, or global improvement. No studies reported the effect of traction on return to work. For CLBP without sciatica, there was moderate-quality evidence that traction makes any difference in pain intensity when compared with sham treatment. No studies reported on the effect of traction on functional status, global improvement or return to work. Adverse effects were reported in seven of the 32 studies which included increased pain, aggravation of neurological signs and subsequent surgery. Four studies reported that there were no adverse effects. The remaining studies did not mention adverse effects. The authors concluded that traction, either alone or in combination with other treatments, has little or no impact on pain intensity, functional status, global improvement and return to work among people with LBP. The authors state that the use of traction as treatment for non-specific LBP is not supported by the best available evidence. Traction is no better than standard interventions for (acute, subacute, and chronic) LBP. They also noted that few participants were identified for any of the principal outcome measurements and, as a result, none of the findings should be considered robust. These conclusions are applicable to both manual and mechanical traction. Further research with RCTs is needed to validate these findings.

Apfel et al. (2010) conducted a retrospective case series of 30 patients with CLBP attributed to disc herniation and/or discogenic LBP. All patients underwent 6-weeks of motorized NSD with the DRX9000. The main outcomes were changes in pain as measured on a verbal rating scale from 0 to 10 during a flexion-extension, ROM evaluation and changes in disc height as measured on CT scans. LBP decreased from 6.2 (±2.2) to 1.6 (±2.3) and disc height increased from 7.5 (±1.7) to 8.8 (±1.7) mm. The authors concluded that NSD was associated with a reduction in pain and an increase in disc height; however, they note that a randomized control is needed to confirm these results. The study is further limited by lack of a control group, lack of long-term follow-up and small sample size.

Schimmel et al. (2009) conducted an RCT of 60 patients to evaluate the efficacy of Intervertebral Differential Dynamics Therapy® (IDD) on LBP vs. sham therapy. Both groups received 20 sessions in the Accu-SPINA device. The IDD group received traction weight that was systematically increased until 50% of a person's body weight plus 4.45 kg (10 lb.) was reached. The SHAM group received a non-therapeutic traction weight of 4.45 kg in all sessions. Outcomes were measured using VAS, ODI, and Short-Form 36 (SF-36) 2, 6 and 14 weeks after initiation of treatment. VAS improved from 61 (+/-25) to 32 (+/-27) in the IDD group and from 53 (+/-26) to 36 (+/-27) in the SHAM group. Leg pain, ODI and SF-36 scores improved in both groups. The authors found no difference between the IDD Therapy and the SHAM therapy; however, patients in both groups reported a decrease in low back and leg pain and an increase in functional status and QOL.

An RCT by Unlu et al. (2008) compared the use of motorized traction, ultrasound, and low-power laser (LPL) therapies in 60 patients (equally distributed) with acute leg pain and LBP caused by lumbar disc herniation. Treatment consisted of 15 sessions over a three-week period. All patients had pre- and post-treatment MRI. Additional outcomes measurements included physical examination of the lumbar spine, VAS, Roland Disability Questionnaire and Modified Oswestry Disability Questionnaire to evaluate functional disability at baseline, after each session, and at one and three-months after treatment. The authors reported similar improvement across treatment conditions for the outcomes measured (pain intensity and functional disability) at the end of the three-week treatment period, and at one and three-month follow-up assessments. Additionally, there were similar reductions in disc herniation on post-treatment MRI evaluations. The authors concluded that all the modalities were effective in the treatment of these patients with acute lumbar disc herniation. The study is limited by lack of a comparison group that did not receive treatment for similar complaints and small sample size.

In a retrospective chart audit by Macario et al. (2008), 100 outpatients with discogenic LBP lasting more than 12 weeks were treated with a 20-month course of motorized spinal decompression via the DRX9000. Overall, this preliminary

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analysis suggests that treatment with the DRX9000 NSD system reduced patient's CLBP with patients requiring fewer analgesics and achieving better function. However, without control groups, it is difficult to know how much of the benefit was placebo, spontaneous recovery, or the treatment itself. Randomized double-blind trials are needed to measure the efficacy of such systems.

Beattie et al. (2008) conducted a prospective case series study of 296 patients to examine outcomes after administration of a prone lumbar traction protocol, using the VAX-D system. All patients had LBP with evidence of a degenerative and/or herniated intervertebral disk at one or more levels of the lumbar spine. Patients involved in litigation or and those receiving workers' compensation were excluded. Patients underwent an 8-week course of prone lumbar traction consisting of five 30-minute sessions a week for 4 weeks, followed by one 30-min session a week for four additional weeks. The numeric pain rating scale and the Roland-Morris Disability Questionnaire were completed at pre-intervention, discharge (within two weeks of the last visit), and at 30 days and 180 days after discharge. Intention-to-treat strategies were used to account for those patients lost to follow-up. A total of 250 (84.4 %) patients completed the treatment protocol with 247 (83.4%) of patients available on 30-day follow-up and 241 (81.4%) patients available at 180-day follow-up. The researchers noted significant improvements for all post-intervention outcome scores when compared with pre-intervention scores ($p < 0.01$). The authors concluded that causal relationships between the outcomes and the intervention cannot be made until further study is performed using randomized comparison groups.

Macario et al. (2006) completed a systematic review of the literature to assess the efficacy of nonsurgical spinal decompression achieved with motorized traction for chronic discogenic lumbosacral back pain. The authors found that the efficacy of spinal decompression achieved with motorized traction for chronic discogenic LBP remains unproven. This may be, in part, due to heterogeneous patient groups and the difficulties involved in properly blinding patients to the mechanical pulling mechanism. Randomized double-blind trials are needed to measure the efficacy of such systems.

Neck

Published clinical evidence for treating neck pain with vertebral axial decompression or other types of motorized traction is limited to case studies. Well-designed RCTs are needed to determine the efficacy of vertebral axial decompression for this indication.

Clinical Practice Guidelines

American College of Physicians (ACP)

In an updated clinical practice guideline on non-invasive treatments for LBP, the ACP (Qaseem et al., 2017) states that evidence is insufficient to determine the effectiveness of several therapies including traction, for acute, subacute, or CLBP. Low-quality evidence showed no clear differences between traction and other active treatments, between traction with physiotherapy versus physiotherapy alone, or between different types of traction in patients with LBP with or without radiculopathy.

North American Spine Society (NASS)

The NASS evidenced based guideline (Kriener et al., 2020; updated 2021) on the diagnosis and treatment for LBP considers the evidence to be insufficient to recommend the use of traction for patients with subacute or CLBP.

The NASS evidence-based guideline (Kriener et al., 2011) on the diagnosis and treatment of degenerative lumbar spinal stenosis considers the evidence to be insufficient to recommend the use of any type of traction in the treatment of lumbar disc herniation with radiculopathy, and lumbar spinal stenosis.

The NASS evidence-based guideline (Bono et al., 2011) on the diagnosis and treatment of cervical radiculopathy from degenerative disorders recommends that future outcome studies for patients in this population treated only with ancillary treatments (such as traction) should include subgroup analysis.

U.S. Food and Drug Administration (FDA)

This section is to be used for informational purposes only. FDA approval alone is not a basis for coverage.

Powered traction equipment is regulated by the FDA, but products are too numerous to list. Refer to the following website for more information (product code ITH): http://www.accessdata.fda.gov/scripts/cdrh/cfdocs/cfPMN/pmn.cfm. (Accessed February 12, 2025)

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Policy History/Revision Information

Date	Summary of Changes
01/01/2026	**Template Update** • Created shared policy version to support application to Oxford plan membership **Supporting Information** • Archived previous policy version 2025T0546V and REHABILITATION 035.19

Instructions for Use

This Medical Policy provides assistance in interpreting UnitedHealthcare standard benefit plans. When deciding coverage, the member specific benefit plan document must be referenced as the terms of the member specific benefit plan may differ from the standard plan. In the event of a conflict, the member specific benefit plan document governs. Before using this policy, check the member specific benefit plan document and any applicable federal or state mandates. UnitedHealthcare reserves the right to modify its Policies and Guidelines, as necessary. This Medical Policy is provided for informational purposes. It does not constitute medical advice.

This Medical Policy may also be applied to Medicare Advantage plans in certain instances. In the absence of a Medicare National Coverage Determination (NCD), Local Coverage Determination (LCD), or other Medicare coverage guidance, CMS allows a Medicare Advantage Organization (MAO) to create its own coverage determinations, using objective evidence-based rationale relying on authoritative evidence (Medicare IOM Pub. No. 100-16, Ch. 4, §90.5).

UnitedHealthcare may also use tools developed by third parties, such as the InterQual® criteria, to assist us in administering health benefits. UnitedHealthcare Medical Policies are intended to be used in connection with the independent professional medical judgment of a qualified health care provider and do not constitute the practice of medicine or medical advice.

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